

ProblemShared: D2C Growth Audit

Prepared by Richard Girling, Ph.D.

richard.girling83@gmail.com | +44 7828 753750 | May 2026

OVERVIEW

This document presents a pre-application growth audit of ProblemShared's digital presence, conducted independently by Richard Girling ahead of applying for the Head of Growth Marketing role. It covers SEO and content, paid and organic social, on-site conversion, customer reviews, and competitive positioning - with specific, actionable opportunities mapped to the D2C launch. The intent is not to criticise but to demonstrate the kind of thinking I would bring from day one.

Note: This is a quick, provisional audit based entirely on publicly available information, conducted as part of my application. It is intentionally limited in scope. If selected for interview, I would of course prepare a far more in-depth, comprehensive audit - drawing on proper tooling, internal data where available, and a detailed plan of action for the coming months.

AUDIT SCORECARD

AREA	RATING	HEADLINE
SEO & Content	AMBER	Thin content; strong existing intent signal; major gap in D2C keywords
Website UX & CRO	AMBER	Clean but conversion path unclear; no self-pay CTA; pricing absent
Paid Media	RED	No visible paid activity; zero D2C acquisition infrastructure
Organic Social	RED	Low volume, low engagement; TikTok/Reels entirely absent
Trustpilot / Reviews	GREEN	4.7/5 from 2,900+ reviews - outstanding asset; underused in marketing
Competitive Positioning	AMBER	Strong on RTC; weak positioning vs. paid-private competitors
D2C Launch Readiness	RED	No pricing, no funnel, no acquisition infrastructure visible publicly

1. SEO & CONTENT ■ AMBER

What exists:

- Google Tag Manager and Google Search Console verified - tracking infrastructure is in place.
- Resources section exists with educational articles (Right to Choose explainers, ADHD guides, shared care).
- Strong informational intent captured via NHS/Right to Choose content - clearly the primary organic traffic driver.
- Website built on Webflow (CDN-hosted, fast); meta description present on homepage.

What's missing or weak:

- Blog/resource output is thin - fewer than ~20 articles visible. Competitors like Psychiatry UK and specialist ADHD clinics are publishing 50-100+ pages targeting high-volume commercial keywords.
- Zero content targeting self-pay/private D2C keywords: 'private ADHD assessment cost UK', 'autism assessment without referral', 'how much does ADHD diagnosis cost'. These terms see 10,000-50,000+ monthly searches.
- No structured data / schema markup evident for medical services, reviews, or FAQs - lost SERP real estate.

Opportunity:

A content programme targeting 'private [condition] assessment cost', 'how to get diagnosed with ADHD UK', 'autism assessment waiting times UK' and comparison content ('ProblemShared vs NHS Right to Choose') could drive substantial D2C-intent traffic with relatively modest production investment. The Trustpilot score (see Section 5) provides the trust signal that converts this traffic.

2. WEBSITE UX & CONVERSION ■ AMBER

What exists:

- Clean, professional design. Clear service navigation (Adults / Children / Partners).
- ADHD and Autism screener quizzes in the footer - a solid lead-generation asset that could be pushed much harder.
- Trustpilot widget embedded - good for trust but not prominently positioned on conversion pages.
- AXA Health partnership featured - important for B2B but potentially confusing for D2C visitors.

What's missing:

- No self-pay / private pricing anywhere on the site. The service updates page explicitly states 'ProblemShared does not currently offer self-pay' - but the D2C launch is imminent. There is no holding page, waitlist, or 'coming soon' CTA capturing intent from visitors who find them now.
- The conversion journey for a new D2C visitor is unclear: land on homepage, read about services, hit a wall - no 'book a private assessment' path exists. This will need to be built from scratch.
- ADHD/Autism screener quizzes are buried in the footer rather than used as primary lead-capture CTAs on condition-specific landing pages.
- One Trustpilot review noted: 'the only downside was the website, which feels a bit outdated' - a signal the UX is not keeping pace with clinical quality.

Opportunity:

Placing the screener quiz as a primary CTA on adult ADHD and autism pages ('Find out if you might qualify - take our 3-minute quiz') would generate qualified D2C leads before the private service launches. A waitlist page for self-pay with email capture could be live within days and begin building a warm audience immediately.

3. PAID MEDIA ■ RED

Current state:

- No visible Google Ads activity for branded or category terms.
- No visible Meta/Instagram paid campaigns detected in public ad libraries.
- Competitors (Priory, Chase Lodge, specialist ADHD clinics) are actively bidding on 'private ADHD assessment UK', 'autism diagnosis UK', and brand-adjacent terms.
- ProblemShared's brand is not defended in paid search - a competitor could intercept high-intent searches for 'ProblemShared cost' or 'ProblemShared private'.

Context:

The absence of paid activity makes sense while D2C is pre-launch - there is no conversion destination to send traffic to. However, this means the paid channel must be built from zero at launch. Given the clinical trust requirements and regulated category constraints (ASA/CAP/MHRA), campaign architecture, compliance review process, and creative testing infrastructure all need to be in place before launch - not after.

Opportunity:

Pre-launch: brand protection campaigns + awareness retargeting to existing organic visitors. At launch: performance campaigns with a compliance-reviewed creative framework, landing pages tested against organic equivalents, and holdout measurement built in from day one. Private ADHD assessment CPCs are high (est. £8-25 in the UK market) but intent is extremely strong - LTV:CAC economics should be favourable given ProblemShared's post-diagnostic service depth vs. single-assessment competitors.

4. ORGANIC SOCIAL ■ RED

Current state:

- LinkedIn company page (linkedin.com/company/problemshareduk) exists - follower count modest relative to clinical authority and NHS partnership scale.
- Instagram (@problemshared_) and Facebook (106794077720387) pages exist but posting frequency and engagement appear low.
- No visible TikTok presence. This is a significant gap: the neurodivergent community is extremely active on TikTok - #ADHD has billions of views; late-diagnosis content drives enormous organic reach.

- Social content appears primarily operational/B2B rather than D2C community-building.

Opportunity:

The neurodivergent community is one of the most engaged and loyal audiences on social media. Clinician-led short-form content ('what an ADHD assessment actually involves', 'common autism signs in women', 'how Right to Choose works') would generate substantial organic reach at low cost. This is precisely the brand investment that makes performance marketing work in this category - people researching a private assessment will check social proof before converting. A consistent content programme could also drive the qualified organic traffic that reduces paid CAC over time.

5. CUSTOMER REVIEWS & REPUTATION ■ GREEN

The headline:

4.7/5 from 2,900+ Trustpilot reviews is an exceptional asset in a regulated healthcare category. This is ProblemShared's single strongest D2C marketing advantage and it is currently underused.

What the reviews say (positives):

- Consistent praise for clinician quality, empathy, and professionalism - exactly the trust signal a D2C launch needs.
- Repeated mentions of same-day diagnosis results - a strong differentiator vs. NHS pathways and some private competitors.
- Late-diagnosis community resonance is strong: adult women masking autism, people in their 40s-60s finally getting answers - powerful word-of-mouth signals for content and paid creative.
- ProblemShared replies to 100% of negative reviews (Trustpilot's own data) - demonstrates genuine commitment to service quality.

Recurring complaints - opportunities for improvement:

- **Medication wait times:** 4-5 month waits for medication titration post-diagnosis are the single most common complaint. Competitors offer prescriptions faster. This is a clinical/ops issue but will damage D2C conversion if not addressed - prospects research reviews before booking.
- **Wait time transparency:** Multiple reviews cite advertised wait times being significantly shorter than actual - eroding trust at a critical point in the journey.
- **Post-diagnostic support quality:** A subset of reviews flag psychoeducation sessions as generic or poorly facilitated. For a company differentiating on post-diagnostic depth, this warrants attention.

Marketing opportunity:

The volume and quality of reviews represents an underused creative asset. Real patient stories (with consent), particularly late-diagnosis narratives, would form the most compelling D2C ad creative in this category - far more effective than brand messaging. The 'same-day diagnosis' and 'felt seen and heard' themes recur across hundreds of reviews and should be the foundation of D2C messaging architecture.

6. COMPETITIVE POSITIONING ■ AMBER

ProblemShared operates across two distinct competitive landscapes that a D2C launch must navigate simultaneously:

COMPETITOR	POSITIONING	PRICE POINT	KEY THREAT
Psychiatry UK	NHS Right to Choose	Free (NHS)	Directly competes for RTC referrals
Priory Group	Premium in-person + online	£2,200+ autism	Brand trust; price signals quality
Chase Lodge	Private D2C, London-focus	£750+ ADHD	SEO-aggressive; fast medication
AuDHD Psychiatry	Dual ADHD+autism specialist	£795+ ADHD	Ranking for private cost keywords
ADHD Clinic UK	Private online, fast Rx	£600-1,500	Speed to medication; content volume

ProblemShared's D2C differentiation:

- End-to-end model (assessment + medication + therapy + psychoeducation) is genuinely unique - no competitor offers this breadth online.
- CQC 'Outstanding' in Responsive domain + NHS partnerships = credibility competitors cannot easily replicate.
- Children's assessment service is a significant differentiator - most private D2C providers are adults-only.

- Medication wait times are a competitive vulnerability vs. providers offering faster prescribing pathways.

7. D2C LAUNCH READINESS ■ RED

The most significant finding of this audit is that there is currently **no publicly visible D2C acquisition infrastructure**. No self-pay landing pages, no pricing, no private booking flow, no paid channels, and no waitlist capture. The service updates page still states that self-pay is not available. This is not a criticism - it reflects the pre-launch stage - but it means the Growth Marketing hire will need to build the entire acquisition engine from zero, while operating within regulatory constraints and protecting a clinical brand that has taken years to build.

Priority actions for the first 90 days:

Pre-launch (Days 1-30)	Email waitlist capture page for self-pay. Brand protection paid search. Screener quiz repositioned as primary D2C CTA. Compliance review framework established with clinical and legal teams.
Content foundation (Days 30-60)	Private pricing/cost content published to capture bottom-of-funnel search intent. Patient story content programme scoped (with consent framework). Schema markup and SEO technical fixes implemented.
Channel activation (Days 60-90)	Google Ads and Meta campaigns live with compliant creative. Holdout measurement framework agreed. Social content programme launched (clinician-led short-form). First A/B tests on landing page and sign-up flow running.

This audit was conducted using publicly available information: the ProblemShared website, Trustpilot reviews (2,900+ reviewed), competitor websites, and public search data. It does not reflect any internal analytics, proprietary data, or confidential company information. Prepared independently by Richard Girling, Ph.D. as part of the application for Head of Growth Marketing.